

Prior Authorization Request

Health Plan

Humana Medicare Advantage -- Gold Choice PPO.

Member Information

Member: Russell K. Haverty DOB: 1959-02-14 (age 67) Member ID: MBR-63390

Sex: M

Requesting Provider

Practice: Ironwood Heart Failure & Device Clinic

Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)

CPT 33249

Planned admit date: 2026-10-08

Level of care requested: INPATIENT

Made-up data for a software prototype. Not a real patient.

Heart failure clinic note

IRONWOOD HEART FAILURE & DEVICE CLINIC

Patient: Haverty, Russell K. Visit: 09/24/2026 Provider: Elena Szymanski, MD

Subjective:

67M w/ NICM dx'd during 07/2026 admission (new onset HF, nonobstructive cors, LGE pattern c/w NICM). Feels better than July. Can walk ~2 blocks on flat before stopping, more SOB on hills. No PND. Using CPAP nightly. Wt stable. No dizziness on current doses.

Objective: BP 108/70, HR 66, Wt 247 lb (BMI 36). JVP not elevated. No edema.

TTE today: LVEF 26% (see report).

Assessment / Plan:

HFrEF, NICM - NYHA class II. GDMT: ARNI, BB, MRA, SGLT2i all at tolerated doses since discharge. LVEF persistently $\leq 35\%$ on today's echo.

-> Referred to EP (Dr. Kowalczyk) for primary prevention ICD; pt wants to proceed and EP has a date in early Oct.

-> SDM visit with decision aid to be completed before implant.

OSA - on CPAP, compliant.

HTN - controlled; amlodipine stopped in July when ARNI started.

CKD 3a - Cr 1.4.

E. Szymanski MD

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Hospital discharge summary

ST. ADALBERT MEDICAL CENTER

DISCHARGE SUMMARY Admitted 07/09/2026 Discharged 07/12/2026

Pt: Haverty, Russell 67 M

Dx: Acute decompensated HF w/ reduced EF, new onset. Cardiomyopathy, non-ischemic.

Course: Presented w/ 2 wks progressive DOE, orthopnea, 15 lb wt gain. BNP 3,900. Diuresed IV furosemide, net -7 L.

LHC 07/10: nonobstructive CAD (RCA 30%, LAD luminal irregularities). No intervention. No prior MI hx.

CMR 07/11: LVEF 24%, LVEDV index 138 mL/m², mid-wall septal LGE; no subendocardial or transmural LGE. Pattern consistent with dilated NICM.

Meds at discharge (new): sacubitril-valsartan, metoprolol succinate, spironolactone, dapagliflozin - all initiated 07/12/2026 at discharge after diuresis complete. Amlodipine discontinued.

F/u HF clinic 1 wk.

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Echocardiogram report

Ironwood Heart - Noninvasive Lab

TTE Date: 09/24/2026 Pt: HAVERTY, RUSSELL K DOB 02/14/1959

Quality: technically difficult (body habitus), contrast used.

LV EF (biplane, contrast): 26%

LVIDd 6.6 cm, global hypokinesis, no regional pattern

RV mildly dilated, fxn low-normal

Mod functional MR

No pericardial effusion

Impression: Severe global LV dysfunction, LVEF 26% (CMR 07/11/26: 24%).

Reader: B. Afolayan, MD

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Medication list with start dates

Current Medications Haverty, Russell K. as of 09/24/2026

- atorvastatin 20 mg daily started 03/15/2020
- sacubitril/valsartan 49/51 mg BID started 07/12/2026 (titrated 08/09)
- metoprolol succinate 50 mg daily started 07/12/2026 (titrated 08/23)
- spironolactone 25 mg daily started 07/12/2026
- dapagliflozin 10 mg daily started 07/12/2026
- furosemide 20 mg daily started 07/12/2026
- hydrochlorothiazide 12.5 mg 02/2018 - stopped 07/09/2026
- amlodipine 10 mg daily 06/2018 - stopped 07/12/2026

Allergies: NKDA

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Shared decision making visit

Ironwood Heart Failure & Device Clinic

ENCOUNTER: Shared decision-making, ICD 09/30/2026

Pt: Russell Haverty (alone, wife on phone)

Clinician: Daniel Oyelaran, RN, MSN, CHFNP

Used an evidence-based ICD decision aid (online interactive version, completed in clinic on tablet, printout attached).

Discussed: chance of benefit vs no ICD, shocks (appropriate/inappropriate), procedure risks, device replacement, end-of-life deactivation, wearable defib as interim alternative.

Pt verbalizes understanding. Most important to him: returning to work part-time as a machinist.

Pt prefers ICD. Questions answered.

D. Oyelaran RN

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EP pre-procedure H&P

EP PRE-PROCEDURE H&P

Date: 10/01/2026 Patient: Russell K. Haverty Admit: 10/08/2026 (inpatient)

Operator: Tomasz Kowalczyk, MD

Procedure: single chamber transvenous ICD, CPT 33249.

Indication: Primary prevention. NICM, LVEF 26% (TTE 9/24), NYHA II, on GDMT since July hospitalization.

No prior MI, PCI or CABG.

Expected LOS 2 midnights. Reasons: severe OSA requiring CPAP monitoring after sedation; BMI 36 with prior difficult IV access; CKD 3a requiring post-procedure BMP.

Contraindication review: no dementia, no cancer or other condition limiting life expectancy to under a year, no AF, hemodynamically stable outpatient. None identified.

T. Kowalczyk MD

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Laboratory results

IRONWOOD LAB - collected 09/24/2026

HAVERTY, RUSSELL

Sodium 137 | Potassium 4.7 | Chloride 101 | CO2 26

BUN 22 | Creatinine 1.4 (H) | eGFR 54 (L)

Hemoglobin 14.1 | Platelets 232

NT-proBNP 1,104 (H)

INR 1.0

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